

PREOPERATIVE MEDICAL NOTES

Member ID:	MA-000004	DOB:	April 18, 1952
Patient Name:	Walter Pryor	Age:	71 years
Line of Business:	Plan MA	Preop Date:	June 5, 2024

CHIEF COMPLAINT

Right knee pain and functional limitation – admitted for total knee arthroplasty

HISTORY OF PRESENT ILLNESS

Patient is a 71-year-old male presenting for preoperative evaluation prior to scheduled total knee arthroplasty on June 10, 2024. Patient reports chronic right knee pain that has progressively worsened over the past 18 months. He describes mechanical symptoms including catching and clicking. Conservative management including physical therapy, NSAIDs, and intra-articular corticosteroid injections over the past 6 months has provided minimal relief. Patient is motivated for surgical intervention and understands the risks and benefits of total knee arthroplasty.

PAST MEDICAL HISTORY

Hypertension, Type 2 Diabetes Mellitus, Hyperlipidemia. Prior left knee arthroscopy 8 years ago. Allergies: NKDA.

PHYSICAL EXAMINATION

Vital Signs:	BP 138/82, HR 76, RR 16, Temp 98.6F
Right Knee Exam:	Mild effusion, tenderness over medial joint line, crepitus with ROM
ROM:	Extension 0°, Flexion 110° (reduced)
Ligamentous:	ACL/PCL intact, collateral ligaments stable

IMAGING STUDIES

Right Knee Radiographs (June 6, 2024): Kellgren-Lawrence Grade IV osteoarthritis with severe joint space narrowing medially and laterally, bone-on-bone articulation, large osteophytes, and subchondral sclerosis. Findings support indication for total knee arthroplasty.

ASSESSMENT

Diagnosis: Unilateral primary osteoarthritis, right knee (M17.11), Grade IV severity, refractory to conservative management.

Surgical Indication: Patient is a good surgical candidate for total knee arthroplasty. Preoperative risk is low. Patient is cleared for surgery.

Preoperative Assessment Completed: June 6, 2024 | Cleared for Total Knee Arthroplasty June 10, 2024